PRACTICE EXAM 08 / EXAM SET 02

Domain 1: Foundational Knowledge and Basic Science

15 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | Health Care Systems and Settings

> **SCENARIO** An outpatient primary care practice assigns patients to a continuing care team. The team tracks referrals, exchanges information with specialists, offers access to advice between visits, and includes patients in care goals.

Which model of care is best illustrated by this organization?

A. A hospital observation unit focused on short-term monitoring.
B. A walk-in service organized around separate, episodic encounters.
C. A diagnostic center organized around individual imaging studies.
D. A patient-centered medical home organized around coordinated primary care.

Q02 | Medical Terminology

Which breakdown correctly explains the term hypoglycemia?

A. Hyper- means below; glyc means fat; -emia means a blood condition.
B. Hypo- means below; glyc means sugar; -emia means a blood condition.
C. Hypo- means below; glyc means sugar; -uria means a blood condition.
D. Hyper- means above; glyc means sugar; -emia means a urine condition.

Q03 | Basic Pharmacology

SCENARIO A verified provider order specifies prednisone 0.03 g orally once now for an adult patient. The clinic stocks 10 mg tablets. The medical assistant is trained and authorized to prepare and administer this medication under applicable law and clinic policy. Use 1 g = 1,000 mg.

How many tablets correspond to the ordered dose? No rounding is needed.

A. 3 tablets
B. 0.3 tablet
C. 30 tablets
D. 300 tablets

Q04 | Psychology

SCENARIO A patient tells the assistant, "I am furious that my insurance company denied my request." Moments later, the patient sharply criticizes the receptionist for a small delay unrelated to the denial.

Which defense mechanism is most consistent with directing the anger toward the receptionist?

A. Denial.
B. Intellectualization.
C. Displacement.
D. Projection.

Q05 | Basic Pharmacology

SCENARIO A patient's medication list contains oral metoprolol and timolol eye drops. A coworker proposes removing the eye drops from the reconciliation list because they act only in the eye. The patient has no new symptoms.

Which explanation best supports keeping both medicines on the list and flagging the combination for clinician review?

A. Ophthalmic timolol can be absorbed systemically and add to the beta-blocking effects of oral metoprolol.
B. Ophthalmic timolol prevents intestinal absorption of metoprolol, making the oral dose ineffective.
C. The ophthalmic route prevents systemic effects, but both products should be listed for refill tracking.
D. The two medicines have opposing beta-receptor effects that reduce the chance of a systemic interaction.

Q06 | Health Care Systems and Settings

SCENARIO An older adult brings several prescription and nonprescription medication bottles to a primary care visit. The clinician requests a detailed review of active ingredients, potential duplicate products, and drug interactions by the team member with medication-specialist expertise.

Which professional is best suited to perform that specialized review?

A. The physical therapist.
B. The registered dietitian.
C. The medical laboratory scientist.
D. The pharmacist.

Q07 | Nutrition

SCENARIO A patient's existing diabetes nutrition plan says to count total carbohydrate and not subtract fiber. One serving of a cereal lists total carbohydrate 24 g, including dietary fiber 3 g and total sugars 4 g. The patient will eat one serving.

Which label value should the assistant help the patient use for carbohydrate counting under this plan?

A. 4 g, using total sugars.
B. 7 g, adding fiber and sugars.
C. 21 g, subtracting fiber from total carbohydrate.
D. 24 g, using total carbohydrate.

Q08 | Medical Terminology

SCENARIO A patient points to a tender area just to the right of the sternum, using the patient's own right side. The draft note incorrectly describes the area as medial to the sternum.

Which wording correctly describes the side and relationship to the body's midline?

A. On the patient's left, lateral to the sternum.
B. On the patient's right, medial to the sternum.
C. On the patient's right, lateral to the sternum.
D. On the patient's right, inferior to the sternum.

Q09 | Health Care Systems and Settings

SCENARIO An employee's national CCMA certification has expired. A new human resources coordinator suggests asking the state medical licensing board to renew the CCMA credential. The employee needs information about the proper renewal process.

Which correction accurately distinguishes certification from licensure?

A. Contact the CCMA certifying organization about renewal; governmental licensure is a separate process.
B. Ask the medical board to convert the expired CCMA credential into a physician license.
C. Use the supervising clinician's active state license in place of renewing the CCMA credential.
D. Ask the clinic to issue a new national CCMA credential based on the employee's work record.

Q10 | Basic Pharmacology

SCENARIO An adult patient with decision-making capacity understands the explanation of a prescribed oral medication and says, "I do not want to take this dose today." The assistant is trained and authorized for medication duties under applicable law and clinic policy. The patient is not in an emergency, and no special legal treatment authority applies.

Which response respects the patient's medication-administration rights?

A. Record the dose as administered because an active prescription is present.
B. Respect the refusal, document it accurately, and notify the prescribing clinician.
C. Place the dose in food without identifying it so the treatment can continue.
D. Ask a family member to authorize the dose instead of accepting the patient's decision.

Q11 | Medical Terminology

SCENARIO A draft medication-history note uses the abbreviation MS without a full medication name. The clinic's terminology policy identifies MS as ambiguous because it may be read as morphine sulfate or magnesium sulfate. The original signed medication order is available for verification.

What should the assistant do before replacing the abbreviation with a full medication name?

A. Expand it to morphine sulfate because that is a familiar meaning in medication histories.
B. Expand it to magnesium sulfate because spelling out either meaning removes the abbreviation.
C. Verify the intended drug against the signed order and seek prescriber clarification if uncertainty remains.
D. Keep MS in the final note so the next reader can select the meaning from the visit context.

Q12 | Basic Pharmacology

SCENARIO An oral medicine's reference states that a substantial fraction is metabolized in the intestinal wall and liver before reaching the systemic circulation. A patient asks why the amount swallowed is not the same as the amount that initially reaches the rest of the body.

Which process best explains the difference described in this reference?

A. Renal excretion of unchanged drug after it enters the systemic circulation.
B. Distribution of drug from the bloodstream into tissues after systemic entry.
C. First-pass metabolism that reduces the amount reaching the systemic circulation.
D. Incomplete gastrointestinal absorption that leaves part of the dose within the digestive tract.

Q13 | Nutrition

SCENARIO Before a scheduled thyroid blood test, a patient mentions taking a high-dose hair-and-nail supplement containing biotin. The supplement is absent from the current list. The laboratory method and any supplement interruption instructions have not yet been checked.

Which action best addresses the supplement-related concern?

A. Advise a standard 24-hour interruption for thyroid testing without checking the assay used.
B. Record the product, dose, and last intake, and alert the clinician or laboratory to possible assay interference.
C. Record the product only in dietary history and proceed without updating the supplement list.
D. Request another thyroid test after the first result returns without informing the testing laboratory.

Q14 | Psychology

SCENARIO At a routine visit, a 16-year-old says, "Everyone talks about my health goals without asking what I want to work on." The care team has already identified two appropriate goals for today, and either can be discussed first.

Which approach best supports the developmental need for increasing autonomy?

A. Have the parent select the first goal before inviting the adolescent to participate.
B. Present both goals as a fixed checklist that the adolescent should follow in the listed order.
C. Postpone discussion of either goal until the adolescent is old enough to direct the entire visit.
D. Invite the adolescent to choose which of the two agreed goals to discuss first.

Q15 | Basic Pharmacology

Which medication class includes albuterol used as a rapid-acting inhaled bronchodilator?

A. Short-acting beta2-adrenergic agonists.
B. Inhaled corticosteroids.
C. Leukotriene receptor antagonists.
D. Centrally acting antitussives.

ANSWER KEY & EXPLANATIONS

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01 D	02 B	03 A	04 C	05 A	06 D
07 D	08 C	09 A	10 B	11 C	12 C
13 B	14 D	15 A			

DETAILED EXPLANATIONS

Q01 | Correct answer: D

A patient-centered medical home organized around coordinated primary care.

A patient-centered medical home emphasizes continuing, team-based primary care, patient involvement, and coordination across services. Tracking referrals and maintaining continuity distinguish this model from a service organized around isolated encounters or tests.

Q02 | Correct answer: B

Hypo- means below; glyc means sugar; -emia means a blood condition.

Hypo- indicates below or deficient, glyc refers to sugar, and -emia refers to a blood condition. Together, these parts describe low blood glucose; -uria refers to urine rather than blood.

Q03 | Correct answer: A

3 tablets

Convert the order before calculating the tablet count: 0.03 g x 1,000 mg/g = 30 mg. Then 30 mg / 10 mg per tablet = 3 tablets; treating 30 mg as 30 tablets would give ten times the ordered dose.

Q04 | Correct answer: C

Displacement.

Displacement redirects an emotion from its original source toward another target. Projection would involve attributing one's own feelings to someone else, rather than directing anger about the insurer at the receptionist.

Q05 | Correct answer: A

Ophthalmic timolol can be absorbed systemically and add to the beta-blocking effects of oral metoprolol.

Timolol eye drops can have systemic effects, and use with an oral beta blocker can produce additive beta blockade. Route alone is not a reason to exclude a medicine from reconciliation; the assistant should report the combination rather than independently stopping either product.

Q06 | Correct answer: D

The pharmacist.

A pharmacist has specialized expertise in medication ingredients, duplication, formulations, and interactions. Other team members contribute to care, but a detailed drug-regimen review is the expertise requested here.

Q07 | Correct answer: D

24 g, using total carbohydrate.

The supplied plan explicitly uses total carbohydrate, which is 24 g for the serving. Sugars and fiber are included within that total; counting sugars alone or subtracting fiber would not follow this patient's existing instructions.

Q08 | Correct answer: C

On the patient's right, lateral to the sternum.

Lateral means away from the body's midline, and laterality is described from the patient's perspective. An area just to the patient's right of the sternum is right lateral to it, not medial or necessarily inferior.

Q09 | Correct answer: A

Contact the CCMA certifying organization about renewal; governmental licensure is a separate process.

The organization that awards a certification sets its renewal or reinstatement process. A state licensing board administers governmental licensure, which is distinct from renewing a national CCMA certification; an employer or supervisor's license does not replace that credential.

Q10 | Correct answer: B

Respect the refusal, document it accurately, and notify the prescribing clinician.

A patient able to make the decision may decline a medication in this situation. The assistant should respect and document that choice and notify the clinician; an order does not justify concealing the medication or documenting a dose that was not taken.

Q11 | Correct answer: C

Verify the intended drug against the signed order and seek prescriber clarification if uncertainty remains.

An ambiguous abbreviation cannot be safely expanded by guessing which drug was intended. The original order provides a source for verification, and unresolved uncertainty requires clarification; spelling out the wrong drug would preserve the error in a more definite form.

Q12 | Correct answer: C

First-pass metabolism that reduces the amount reaching the systemic circulation.

First-pass metabolism describes drug metabolism in the intestinal wall or liver before an absorbed oral dose reaches the systemic circulation. Incomplete absorption can also limit systemic availability, but the reference specifically attributes this reduction to metabolism before systemic entry.

Q13 | Correct answer: B

Record the product, dose, and last intake, and alert the clinician or laboratory to possible assay interference.

Biotin can interfere with certain laboratory assays and produce misleading results. The team needs the supplement details and test-specific guidance; the assistant should not invent a universal interruption period or independently interpret a possibly affected result.

Q14 | Correct answer: D

Invite the adolescent to choose which of the two agreed goals to discuss first.

Offering a meaningful choice within agreed clinical goals supports an adolescent's developing autonomy and participation. The assistant can encourage involvement without transferring all clinical decisions to the adolescent or excluding the care team.

Q15 | Correct answer: A

Short-acting beta2-adrenergic agonists.

Albuterol is a short-acting beta2-adrenergic agonist that relaxes airway smooth muscle. It is distinct from inhaled corticosteroids and leukotriene modifiers used for other aspects of airway management, and from antitussives that suppress cough.

End of Practice Exam